Five simulated user scenarios

Ruta Clara | Fernanda Espinosa | Week 13

Evidence boundary

This is an authored design exercise with five fictional profiles and hypothetical reactions. No real participants were recruited, interviewed or observed. All quotations below are invented scenario dialogue, not participant quotes or outputs of five independently executed persona sessions. The required five-person study, including at least three strangers and no classmates, remains pending.

Purpose and common scenario

Explore possible confusion in a fictional referral workflow: patient contacted, institution accepted, appointment confirmed and attendance verified. Each scenario uses invented clinical information. No patient records, real staff identities, task times, success rates, savings or recruitment relationships are claimed.

P1 | Mariana, 52 | Fictional clinic navigator

Profile: Uses paper and WhatsApp, works under time pressure and distrusts unfamiliar systems.

Task: Record contact evidence and identify the next unresolved handoff.

Hypothesis: a general verification label could make contact confirmation look like confirmation of the whole route.

Invented scenario dialogue: "If I confirm this contact, am I also saying the institution will receive her?"

Design response: Use the exact step name in the status and explain that other steps remain pending. This clarification was already implemented after a separate synthetic screenshot walkthrough; P1 is not additional validation.

P2 | Laura, 36 | Fictional clinic receptionist

Profile: Handles appointment calls and prefers short administrative instructions.

Task: Record institutional acceptance after reviewing fictional capacity, documents and cost.

Hypothesis: separate acceptance and route-review fields could feel repetitive, causing her to skip the route or stop.

Invented scenario dialogue: "I entered the acceptance. What else do I need to check before I can continue?"

Design response: Show the missing prerequisite beside the blocked action. A route-review instruction is present in the build; whether it reduces effort needs a real session.

P3 | Ana, 47 | Fictional referral beneficiary

Profile: Relies on a phone and wants to know whether the next appointment is arranged. She is a secondary stakeholder, not the primary navigator user.

Task: Interpret a navigator-shown status summary without editing the case.

Hypothesis: appointment confirmed could be interpreted as the end of the referral rather than a pending attendance step.

Invented scenario dialogue: "Does having the appointment mean everything is finished?"

Design response: Keep attendance visibly separate and label appointment as a scheduled next step. Do not infer that Ana has access to an implemented patient portal; no such portal is claimed.

P4 | Diego, 29 | Fictional administrative assistant

Profile: Relatively comfortable with web forms but unsure what counts as reliable evidence.

Task: Enter a fictional evidence source, recording person and category.

Hypothesis: he may paste a phone number or assume that valid formatting proves the source is true.

Invented scenario dialogue: "What should I write as the source, and who checks whether it is true?"

Design response: Provide a short fictional example, explain the responsible verifier and prohibit patient identifiers. Existing source guidance excludes personal data; stronger provenance and authorized verification remain future requirements for real use.

P5 | Elena, 44 | Fictional referral coordinator

Profile: Checks unresolved cases and wants corrections to leave a clear explanation.

Task: Correct institutional acceptance after later steps have been recorded.

Hypothesis: she may read reopened progress as lost work or a software error, or expect later statuses to remain completed.

Invented scenario dialogue: "Why did the appointment and attendance become pending after I corrected acceptance?"

Design response: Explain which supporting evidence changed and which later steps reopened. The current build reopens dependent steps; finer-grained correction history and comprehension still need testing.

Hypotheses to prioritize

This ranking is an analyst judgment for planning. It is not based on five observed sessions. Scores use 1 (low) to 3 (high); the sum helps compare concerns and does not establish prevalence or clinical risk.

Priority / hypothetical pain	Harm	Blockage	Effort	Total
1. Confusing recorded status with verified care	3	3	2	8
2. Not knowing which evidence or prerequisite is missing	2	3	2	7
3. Interpreting correction as lost progress	2	2	2	6

Proposed first fix

Prioritize step-specific verification language because false reassurance conflicts with the Blueprint shadow clause. Keep recorded activity, verification and received care separate. This wording was already improved in the earlier synthetic screenshot review; these five fictional profiles do not prove that the fix works for people.

Real retest and return action - proposed, not completed

Ask relevant navigators to identify the next action, explain what a confirmed appointment proves, try unsupported attendance evidence, and correct an earlier step. Use neutral prompts and the same fictional cases. Record actual hesitation, completion, help and exact consented quotes; compare matched manual and app tasks including setup and correction effort. Invite a voluntary second attempt on another fictional case and record whether it actually occurs. No return action is claimed here.

Requirements still open

Five real relevant users, at least three strangers and no classmates; observed ranking of three pains; the observed top-pain fix and real retest; recruitment and consent evidence; matched economics and observable return behavior. This supplement satisfies none of those field-study gates.

Submission label

Simulated user study supplement: five fictional scenarios were created to explore possible usability problems. No real participants were recruited or interviewed. The required five-person study remains pending.